Dr. Muhammad Ghani
Dr. Ramalingam Ratnasabapathy
Dr. Restituto Tibayan

2460 W. Horizon Ridge Parkway
Henderson, Nevada 89052
Phone: 702-822-2000
Fax: 702-938-2235

To: A Plus Home Health

From: Daisy 😁 Dr Ram

Fax: 702 342-8398

Pages (Including Cover):

Phone:

Date: 2/26/2026

Re: Lisa Rodriguez

DOB: 12/21/1981

Reply ASAP:

For Your Information:

Home Health

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Comp Cancer Center NV Horizon Ridge
2460 W. Horizon Ridge Parkway
Henderson NV 89052
Phone: 702-822-2000
Fax: 702-938-2235

Patient: RODRIGUEZ, LISA
DOB: 12/21/1981 ID: 1819534
Sex: Female

Date ordered: 02/25/2026
Ordered by : Ram Ratnasabapathy MD

Allergies: iron dextran complex, chlorhexidine

Order: Home health referral
To be done: STAT
Order Instructions: eval. for PT/OT
Associated problems: Primary malignant neoplasm of colon (disorder) * (C18.7)

Appropriate Use Criteria Details:

Ram Ratnasabapathy MD(1144220781)

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COMPREHENSIVE
CANCER CENTERS
2460 W. Horizon Ridge Parkway
Henderson, NV 89052
P: 702-822-2000 F: 702-938-2235

PATIENT: LISA RODRIGUEZ
MRN: 1819534
DATE: 01/21/2026
DOB: 12/21/1981
ATTENDING PHYSICIAN: Ram Ratnasabapathy, MD

FOLLOW-UP NOTE

- Primary malignant neoplasm of colon (disorder) (Stage Date: 10/05/2021, Stage IIIC (T4a, N2a, McM0)-Pathological
  Date of Dx:9/2021 ; ICD-10:C18.7 ;Malignant neoplasm of sigmoid colon)

HPI~
Ms. Rodriguez is a pleasant 43 year old woman with colon cancer. Patient underwent colonoscopy on 9/8/21 and was found to have a mass at 35 cm. Biopsy was positive for adenocarcinoma. She underwent total proctocolectomy and ileostomy on 9/21/21. Pathology was reported as adenocarcinoma, 3.5 x 2 cm, Grade 2, negative margins, negative for lymphatic, vascular and perineural invasion, 6/36 positive nodes, 3 tumor deposits and was staged as T4a N2a. Post op, she developed pneumomediastinum which resolved on its own. She was started on FOLFOX on 10/20/21. She developed left arm pain and left jaw pain on second day of 5-FU infusion. Due to concern for possible coronary vasospasm, her pump was discontinued. Subsequently, she was able to tolerate chemotherapy. She completed 12 cycles on 5/16/22. Oxaliplatin was held in last 2 cycles due to neuropathy. She underwent ileostomy reversal on 7/28/22. She was admitted at Southern Hills hospital on 8/31/22 with bowel obstruction. At that time, she was also found to have a pelvic mass. She was evaluated by Dr Kushnir and underwent surgery at Valley hospital on 9/7/22. She was found to have large pelvic mass in relation to ovary and bowel loops. Biopsy was positive for colon cancer. She was admitted at Southern hills hospital and underwent surgery by Dr Rives on 9/20/22. He was able to resect the mass and performed another ileostomy. Pathology was reported as colon cancer. She was started on FOLFIRI and Bevacizumab on 12/12/22 and she completed 11 cycles. Subsequently, she underwent PET scan and MRI pelvis in 08/23 and was found to have large 13 cm pelvic mass c/w recurrent disease. She underwent hysterectomy left oophorectomy and bilateral salpingectomy on 8/17/23. Pathology did show metastatic colon cancer in left ovary. There were serosal metastatic implants on uterus. The patient underwent evaluation at Mayo Clinics. MRI of abdomen did show single liver metastases and evidence of peritoneal disease. She was started on FOLFOXIRI on 10/16/23. She went to Mayo Clinics in 01/24 and was advised to continue chemotherapy for 3 more months. She underwent HipEC and debulking on 7/11/24 at Mayo Clinic. She was advised to continue FOLFOX and Bevacizumab and she completed 4 cycles. She had MRI done at Mayo Clinics. As per patient, she has been advised to continue on 5-FU/Leucovorin and Bevacizumab. She was found to have stomal obstruction in 11/24 due to tumor and required surgery. PET scan showed disease progression and she was started on Lonsurf and Bevacizumab in 02/25 and could not tolerate it. Subsequently, she was placed on re challenging with FOLFIRINOX and progressed. Patient completed palliative radiation to Abdominal wall metastasis. She is currently on fruquintinib. This is a KRAS G12D mutated progressive colon cancer.

INTERVAL HISTORY~
Interval History
01/21/2026: Patient reports not feeling well since last visit. She discontinued fruquintinib with last dose on 01/14/2026 due to GI bleeding. Patient underwent endoscopic evaluation of stump which revealed stump infection and was diagnosed with ulcerative colitis. She was prescribed antibiotics for inflammation by gastroenterology. Patient continues to have intermittent bleeding episodes and reports ongoing pain.
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REVIEW OF SYSTEMS~
General: no weight loss, no fever, no chills, no night sweats, energy level good.

Eyes: no diplopia, no transient or permanent loss of vision, no scotomata.
ENT/Mouth: no tinnitus, normal hearing, no epistaxis, no hoarseness, no oral ulcers, no gingival bleeding, no sore throat, no postnasal drip, no mouth pain, no sinus pain, no dysphagia.
Cardiovascular: no chest pain, no palpitations, no syncope, no upper extremity edema, no lower extremity edema, no calf discomfort.
Respiratory: no cough, no hemoptysis, no pleurisy, no wheezing.
Gastrointestinal: reports bleeding, no abdominal pain, no nausea, no vomiting, no constipation, no hematochezia, no jaundice, no abnormal cramping, no hematemesis, no diarrhea, no melena, no dyspepsia, no dysphagia.
Musculoskeletal: no muscle pain, no swollen joints redness, no bone pain, no spine tenderness.
Skin: no rash, no nodules, no pruritus, no lesions.
Neurologic: no confusion, no seizures, no syncope, no tremor, no speech change, no headache, no hiccups, no abnormal gait, no weakness, no sensory changes.
Psychiatric: no depression, no anxiety, concentration normal.
Endocrine: no polyuria, no polydipsia, no thyroid disease symptoms, no hot flashes.
Hematologic: no epistaxis, no gingival bleeding, no petechiae, no ecchymosis.
Lymphatic: no lymphadenopathy, no lymphedema.
Allergy/Immunologic: no eczema, no frequent mucous membrane infections, no frequent respiratory infections, no recurrent urticaria, no frequent skin infections.

VITAL SIGNS~
Blood pressure: 100/84, Sitting, R arm, Regular, Pulse: 88, Temperature: 97.9 F, Respirations: 18, O2 sat: 97%, At Rest, Room Air, Pain Scale: 0, Height: 63 in, Weight: 100.2 lb, BSA: 1.44, BMI: 17.75 kg/m2

PHYSICAL EXAM~
General: No pallor. No palpable lymphadenopathy.
Neurological: Alert and oriented to time, place, and person. Pupils equally round and reactive to light. Extraocular movements normal. Gait is normal. Speech is normal.
HEENT: Pharynx normal. No hoarseness. No sinus tenderness.
Ear, nose, and throat exam: Normal.
Lungs: Clear.
Cardiovascular: Regular. No murmur. No cardiomegaly.
Abdomen: Nontender.
Extremities: No pedal edema. No calf tenderness.
Skin: Intact.

LABORATORY FINDINGS~

LabResults	01/21/2026	12/31/2025	12/23/2025	12/19/2025	12/04/2025	11/21/2025
CBC						
WBC x 10^3/uL	4.1 (L)	3.7 (L)			2.9 Critical Value Verified (LL)	3.6 (L)
RBC x 10^6/uL	4.41	4.37			4.90	4.24
HGB g/dL	12.0	11.7			12.3	10.4 (L)
HCT %	37.5	37.4			40.4	34.3
MCV fL	85	86			82	81
MCH pg	27	27			25 (L)	25 (L)
MCHC g/dL	32.0	31.3 (L)			30.4 (L)	30.3 (L)
RDW %	17.2 (H)	20.9 (H)			21.6 (H)	20.3 (H)
PLT x 10^3/uL	138 (L)	136 (L)			110 (L)	132 (L)
Neu %	77 (H)	76 (H)			71	80 (H)
LY %	9 (L)	9 (L)			16 (L)	10 (L)
MO %	8	9			10	7
EO %	4	6			2	2
IG %	0.50	0.30			0.30	0.60
Neu # (ANC) x 10^3/uL	3.2	2.8			2.1 (L)	2.8
BA %	1	1			1	1
MO # x 10^3/uL	0.3 (L)	0.3 (L)			0.3 (L)	0.3 (L)
EO # x 10^3/uL	0.17	0.22 (H)			0.07	0.07

BA # x 10^3/uL	0.0	0.0			0.0	0.0
IG # x 10^3/uL	0.02	0.01			0.01	0.02
LY # x 10^3/uL	0.4 (L)	0.3 (L)			0.5 (L)	0.4 (L)
ManualDifferential						
Chemistries						
Glucose mg/dL		98			95	113 (H)
BUN mg/dL		16			19	15
Creatinine mg/dL		0.7			1.0	0.8
Sodium mmol/L		141			141	143
Potassium mmol/L		3.9			4.2	4.1
Chloride mmol/L		103			101	102
CO2 mmol/L		27			31	30
Calcium mg/dL		9.6			10.0	9.4
Albumin g/dL		4.3			4.6	4.4
Total protein g/dL		7.0			7.6	7.0
Globulin g/dL		2.68			2.97	2.59
A/G ratio		1.6			1.6	1.7
Bilirubin, total mg/dL		0.4			0.6	0.5
Alkaline phosphatase U/L		210 (H)			356 (H)	457 (H)
AST/SGOT U/L		22			46 (H)	38 (H)
ALT/SGPT U/L		14			47	86 (H)
GFR estimate mL/min/1.73m2		102			70	92
Vitamin D, 25-hydroxy ng/mL		32				
TumorMarkers						
CEA ng/mL		301.0 (H)			218.2 (H)	
AnemiaLabs						
Iron ug/dL		49 (L)				
TIBC ug/dL		302				
Ferritin ng/mL		419 (H)				
Iron, % saturation		16				
Vitamin B12 pg/mL		570				
Folate, serum ng/mL		12.98				
Microbiology						
Hormones						
Urine						
Pathology/Cytology						
Pathology report			See attached			
Genetics						
Lab - Other						
Lab Report				See attached		

• 01/21/2026: WBC 4.1, hemoglobin 12.0, platelets 138

IMAGING STUDIES~

CT AP done 12/18/2025: Again seen are dilated loops of bowel filled with fluid in the pelvis. Consider bowel obstruction. Multiple surgical anastomotic staple lines are present in the loops of bowel. There is a right lower quadrant ostomy. Consider adhesions with other etiologies not excluded. This is similar to previous exam. Multiple hypoenhancing masses in the liver and numerous varying size lung nodules are present. These are concerning for metastatic disease. These are similar to previous exam.

NM bone scan done 11/25/2025: No definite bony metastatic disease identified.

PATHOLOGY~
Pathology done 12/23/2025: A. REctum, proximal, biopsy: Rectal mucosa with mild proctitis and reactive/inflammatory changes. Negative for dysplasia or malignancy. B. Rectum, distal, biopsy: Rectal mucosa with mild proctitis and reactive/inflammatory changes. Negative for dysplasia or malignancy.

ASSESSMENT~
1. Ms. Rodriguez is a 44-year-old woman with KRAS G12D mutated, MSI stable, HER2 negative metastatic colorectal cancer, T4a N2a. Disease primarily involves peritoneum, abdominal wall, liver, and subcutaneous tissue. She has been on sequential lines of therapy including FOLFOX, FOLFIRI with Bevacizumab, FOLFOXIRI, and Lonsurf with Bevacizumab. Patient has failed four lines of therapy. Patient started fruquintinib as fifth-line anti-VEGF therapy as bridging treatment until G12D directed clinical trials become available. 01/21/2026: Patient discontinued fruquintinib on 01/14/2026 due to GI bleeding. Current labs show improvement with WBC 4.1, hemoglobin 12.0, platelets 138. Patient scheduled for tertiary care consultation at Mayo Clinic on 02/04/2026 for further treatment options.

2. Iron deficiency anemia with hemoglobin stable at 12.0. Patient previously received iron infusion with continued improvement noted.

3. Mild leukopenia resolved with WBC improved to 4.1 from previous 2.9.

4. Ulcerative colitis with stump inflammation diagnosed by gastroenterology via endoscopy. Patient prescribed antibiotics for treatment.
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PLAN~
1. Discontinue fruquintinib due to GI bleeding and stump complications with ulcerative colitis flare.

2. Initiate steroid therapy for ulcerative colitis flare-up in stump area as recommended by gastroenterology.

3. Plan to restart fruquintinib at reduced frequency of every other day as tolerated after resolution of current bleeding episode.

4. Tertiary care consultation scheduled at Mayo Clinic on 02/04/2026 for input on further treatment options and potential clinical trial enrollment.

5. Continue coordination for G12D directed clinical trial options.

6. Monitor complete blood count and iron studies given history of anemia.

7. Follow up in 2 weeks to reassess treatment tolerance and response to steroid therapy.

Treatment plan is consistent with current NCCN Clinical Practice Guidelines in Oncology.

SUMMARY:
- Patient discontinued fruquintinib on 01/14/2026 due to GI bleeding
- New diagnosis of ulcerative colitis with stump inflammation managed with antibiotics and steroids
- Laboratory values show improvement in white blood cell count and stable hemoglobin
- Tertiary care consultation at Mayo Clinic scheduled for 02/04/2026
- Plan to restart fruquintinib at reduced frequency of every other day as tolerated
- Follow up in 2 weeks to assess treatment tolerance and response
{ }
NEW ORDERS~
Labs:

- 01/21/2026, CBC w/ auto diff, Perform Date: 02/05/2026, Perform Location: Horizon Ridge
- 01/21/2026, CEA panel, Perform Date: 02/05/2026, Perform Location: Horizon Ridge
- 01/21/2026, CMP, Perform Date: 02/05/2026, Perform Location: Horizon Ridge

Services:

- 01/21/2026, Blood draw via port, Perform Date: 01/21/2026
- 01/21/2026, Blood draw via port, Perform Date: 02/05/2026
- 01/21/2026, RTC MD, Perform Date: 02/05/2026, Perform Location: Horizon Ridge

Imaging: None
Prescriptions: None
.

RAM RATNASABAPATHY, MD

Send copy of note to:

This document serves as a record of the services and decisions personally performed and made by Ram Ratnasabapathy, MD. It was created on his behalf by_____, a trained medical scribe. The creation of this document is based on the provider's statements to the medical scribe.

The information in this document, created by the medical scribe for me, accurately reflects the services I personally performed and the decisions made by me. I have reviewed and approved this document for accuracy prior to leaving the patient care area.

Electronically signed by Ram Ratnasabapathy MD 01/21/2026 22:25 PST

Anthem

Lisa Rodriguez

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VFZ745W26118

MEDICARE SUPPLEMENT

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Issue Date: 01/10/2026

MEDICARE HEALTH INSURANCE

Name/Nombre

LISA RODRIGUEZ

Medicare Number/Numero de Medicare

7V25-U99-KG75

Entitled to/Con derecho a	Coverage starts/Cobertura empieza
HOSPITAL (PART A)	03-01-2024
MEDICAL (PART B)	03-01-2024

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1-800-MEDICARE (1-800-633-4227 / TTY: 1-877-486-2048); **Medicare.gov**

Patient Demographics

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Patient: RODRIGUEZ, LISA

DOB: 12/21/1981
Sex: F

Attending MD: Ratnasabapathy, Ram

Practice: Comprehensive Cancer Center of Nevada
Location: Horizon Ridge
Location Address: 2460 W. Horizon Ridge Parkway, Henderson, NV, 89052
Location Phone: 702-822-2000
Location Fax: 702-938-2235
Date/Time Printed: 02/25/2026 15:42 America/Los_Angeles

Patient Information

Field	Details
Address	Home: 7077 SUNLAND HILLS AVE, LAS VEGAS, NV, 89113-4735
Phone	Mobile: (702)336-1676
Email	Primary: 32LISA.3@GMAIL.COM
Preferred Language	English
Race	WHITE White
Next of Kin Contact	N/A

Preferred Pharmacy

Name	Address	Phone	Fax	NCPDP
WALGREENS DRUG STORE #02598	7685 S RAINBOW BLVD, LAS VEGAS, NV 891395477	7026143094	7026143189	2989361
Oncomed DBA Onco360	13410 Eastpoint CenterSuite 101, Louisville, KY 40223	8776626633	8776626355	1834248

Drug Benefit Plan

Name	Coverage Type
HUMANA MEDICARE(DST PHARMACY SOLUTIONS DIRECT)	Patient match alert:NOT_MATCHED Retail:Covered Mail Order:Not Covered

Active Insurances

Insured Name	Type	Effective Date	Expiration Date	Carrier	Policy #	Document Name	Status	Comments
UNKNOWN	Teritiary	11/27/2024	04/23/2026	FINANCIAL HARDSHIP			Active	
RODRIGUEZ, LISA	Other	01/01/2025	09/21/2025	ALIGNMENT HEALTH PLAN / PO BOX 14010	00000357407		Expired	
RODRIGUEZ, LISA	Teritiary	09/22/2025	12/31/2025	ALIGNMENT HEALTH PLAN / PO BOX 14012	00000357407		Expired	
RODRIGUEZ, LISA	Secondary	01/01/2026		ANTHEM BLUE CROSS BLUE	YFZ745W261 18		Active	

RODRIGUEZ, LISA (Sex: Female DOB: 12/21/1981 MRN: 1819534)

Insured Name	Type	Effective Date	Expiration Date	Carrier	Policy #	Document Name	Status	Comments
				NEVADA / PO BOX 60007				
UNKNOWN	Other	11/01/2024	11/01/2024	SELF PAY / NO INSURANCE			Expired	
RODRIGUEZ, LISA	Primary	03/01/2024		MEDICARE PART B NEVADA / PO BOX 6776	7V25U99KG75		Active	
RODRIGUEZ, LISA	Other	03/01/2024	09/30/2024	MEDICAID NEVADA / PO BOX 30042	53953400010		Expired	
RODRIGUEZ, LISA	Other	04/08/2022	02/29/2024	SILVER SUMMIT HEALTHPLAN / PO BOX 5090	53953400010		Expired	
RODRIGUEZ, LISA	Other	01/01/2021	01/31/2024	UNITED HEALTHCARE / PO BOX 740800	968040050		Expired	
RODRIGUEZ, LISA	Other	12/01/2021	12/31/2022	ZIEXTENZO, SANDOZ ONE SOURCE / 2250 PERIMETER PARK DR	29189992410		Expired	
RODRIGUEZ, LISA	Other	04/01/2020	01/31/2021	HEALTH PLAN OF NEVADA INC / PO BOX 15645	20002026100		Expired	